8.4 Hyperosmolar Hyperglycemic State*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family report 2 days of drowsiness **Objective** • Glucose 900 mg/dL • Very high serum osmolality • Few ketones • Dry mouth	**ACTUAL** Inadequate fluid volume related to severe osmotic diuresis as evidenced by dehydration and hyperosmolality.	• Fluid deficit and glucose corrected gradually over 24–48 hours.	• Give IV fluids as prescribed; correction is slower than in DKA. • Follow sodium and osmolality. • Measure urine output hourly. • Follow neurological status.	• Goal reached as hydration and alertness improve.
Subjective • Family say, “She does not recognize us.” **Objective** • Confused • GCS 12	**ACTUAL** Acute confusion related to hyperosmolality as evidenced by drowsiness and confusion.	• Back to her usual orientation as osmolality corrects.	• Check GCS hourly. • Keep her safe: low bed, rails per policy. • Reorient her often.	• Met once she is oriented.
Subjective *No specific complaint* **Objective** • Dehydrated • Immobile older adult	**RISK** Risk for thrombosis as evidenced by risk factors: hyperviscosity and immobility.	• No VTE.	• Give prophylactic anticoagulation as ordered. • Mobilize as soon as possible. • Examine the calves daily.	• Achieved if VTE does not develop.

*References for this care plan: [3], [35], [127], [128]. Full entries are listed in the References section of the CPMN book.